

Diablo Interview Brief

The value package, the story, and the eight numbers that turn it from a claim into evidence.

The ruled blanks are the work that's left. Write your real before-and-after figures into them – an approximate number beats an unquantified claim every time.

THE ONE LINE

“My value is my ability to step into complex, high-volume operations, quickly identify opportunities, develop the team, and implement sustainable workflows that improve performance and the member experience.”

Say it once, early. Everything after this is proof.

OPENING – 90 SECONDS

“Tell me about yourself.”

SAY THIS

I've spent the last three-plus years supervising pharmacy operations in the Diablo service area. I started with three pharmacies, took on a fourth, and the assignment was the same each time: a location that needed to perform better. I'd go in, find where the workflow was breaking, rebuild it with the team, and hold it there.

That's what got me sent to Oakland Main Hospital Pharmacy for two months — the result there. And it's why I was asked to take Shadelands and Walnut Creek, two of the busiest pharmacies we run, filling about Rx/day a day between them. Today I'm responsible for more than fifty people across those sites.

What I bring isn't just experience supervising. It's a repeatable way to walk into a complex, high-volume operation, find the opportunities fast, develop the team, and put workflows in place that hold after I move on.

Ends on the thesis. Do not run past two minutes.

What you're actually selling

SCALE

Proven operational leadership

Three-plus years supervising pharmacy operations across multiple locations and a team of 50+. Not one site, one shift — a portfolio.

TURNAROUND

Performance recovery

You get handed the locations that need improvement. You find the gap, build the workflow, train the team, and the number moves:

$X\% \rightarrow Y\%$.

VOLUME

High-volume execution

Two of the busiest pharmacies in the service area. Call-outs, system outages, a full waiter queue, and an escalated member — at the same time — and decisions still get made in real time.

PEOPLE

Developing the bench

You coach, set expectations, give feedback, and delegate real responsibility, so the operation doesn't stop when the supervisor isn't standing there.

CHANGE

Process improvement

"That's how we've always done it" isn't an answer. You evaluate the workflow, find the bottleneck, change it, measure whether it worked, and adjust when it didn't.

USE IT

How to deploy these

Every behavioral answer should land on one pillar. If an answer doesn't map to one of the five, it's a story you don't need to tell.

The story they'll remember

Told in order, the assignments say what you shouldn't have to say out loud: leadership kept handing you bigger and harder problems.

01

Three pharmacies

First supervisory span. Learned the operation from the floor up.

02

Expanded to four

Workflows rebuilt, team performance improved — what moved.

03

Oakland Main Hospital Pharmacy — two months

Asked to support a struggling site on a defined assignment. Left it at result.

04

Shadelands and Walnut Creek

Same playbook, two of the highest-volume environments in the area.

05

50+ employees today

The span you're operating at right now — and the floor, not the ceiling, of what you're interviewing for.

Eight metrics that carry the whole package

Approximate is fine. “Roughly, we went from about 68% to the low 80s over four months” beats a precise number you can't remember under pressure — and it beats “I improved metrics” by a mile. Fill what you know; leave the rest blank rather than guessing at something they can check.

TME — member experience

before → after in months

Their headline metric. Lead with this one if it moved.

JDP score

before → after in months

External validation — worth naming even if the move was modest.

Wait time / waiter turnaround

before → after by what you changed

The most tangible number in the room. Everyone understands minutes.

Prescription volume

Rx per day/week across # sites

Establishes scale before you claim you can handle scale.

Productivity — Rx per tech hour / per FTE

before → after

Proves the workflow change did something, not just the extra bodies.

Attendance / call-out rate

before → after

The one that shows you manage people, not just process.

Training / competency completion

before → after

Your bench-building pillar, in a number. Compliance-adjacent, so it lands.

Staffing — vacancies, turnover, time-to-fill

before → after

Retention in a high-volume site is a leadership result, and they know it.

BEHAVIORAL ANSWERS

Six questions, already answered

Each one is a skeleton — situation, action, result — keyed to a pillar. Fill the result with a real number and the answer is finished.

“Tell me about a time you turned around an underperforming location.”

SITUATION Oakland Main Hospital Pharmacy. Two-month assignment, sent in because performance needed to improve.

ACTION First week was observation, not changes — where the work stalled, where staff were doubling back, which step created the queue. Then one workflow change at a time, trained on the floor, with the leads owning it.

RESULT metric before → after — and it held after the assignment ended.

PILLAR Performance turnaround.

“Walk me through a day with competing priorities.”

SITUATION Two call-outs, a system outage, a full waiter queue, and an escalated member — one morning, one of the busiest sites in the area.

ACTION Triage in order: safety first, then the member in front of you, then throughput. Redeployed staff to the bottleneck, moved non-urgent work to later in the day, took the escalation yourself so the team stayed on the queue.

RESULT wait time held at / recovered by.

PILLAR High-volume execution.

“Tell me about a difficult performance conversation.”

SITUATION An employee whose attendance or output was affecting the rest of the team — and the team knew it.

ACTION Private, specific, documented. Named the behavior and the impact, not the person. Set a clear expectation with a date, then followed up on that date instead of letting it drift.

RESULT what changed. Say what happened even if the answer was separation — owning that reads as stronger, not weaker.

PILLAR People development.

“Tell me about a change people resisted.”

SITUATION A workflow that had been done the same way for years and was creating the backlog.

ACTION Brought the staff who ran it into the fix instead of announcing it. Piloted it short, measured it, showed them their own numbers, adjusted the part they were right about.

RESULT metric moved, and the team defended the new process to the next site.

PILLAR Change and process improvement.

“Tell me about an escalated member complaint.”

SITUATION A member whose issue had already gone past the staff member in front of them.

ACTION Took it off the counter, listened without defending, fixed what could be fixed that day, and told them exactly what would happen next — then closed the loop with the staff member so it wasn't just handled, it was taught.

RESULT Resolved without further escalation; TME / complaint trend.

PILLAR Member experience.

“Tell me about someone you developed.”

SITUATION A technician or lead with more capability than their role was using.

ACTION Delegated something real with your name still on the outcome. Set the expectation, checked in on a schedule rather than hovering, gave feedback that was specific enough to act on.

RESULT promoted / now runs X — and the site ran without you standing in it.

PILLAR People development.

THE CLOSER

“Why should we hire you?”

SAY THIS

Because you're not taking a risk on whether I can do this — I've already done it here, more than once, at increasing scale. Three pharmacies, then four, then Oakland Main, then the two busiest sites in the service area. Every time, the operation was better when I left it than when I got there: your strongest number.

And I don't do it by being the person everything runs through. I build the team so the performance stays after I move to the next problem. That's what I'd bring to this role.

Questions that make you sound like the peer, not the candidate

- **What does success look like in the first 90 days** for whoever takes this?
- **Which metric is under the most pressure right now** — and what's already been tried?
- **How much authority does this role have to change workflow** directly, versus escalate a recommendation?
- **What's the widest gap** between your strongest and weakest site, and what do you think is driving it?
- **Who would I be partnering with** — pharmacists in charge, ops, HR — and where do those lines actually run day to day?

TRAPS ---

What loses the room

- **"I improved the metrics."** Unquantified claims read as no claim at all. Number, timeframe, mechanism — every time.
- **Describing the problem without the change.** They don't need the diagnosis, they need what you did about it.
- **"We" for everything.** Credit the team, then say the decision that was yours. Both, in that order.
- **The four-minute STAR answer.** Cap it at two. Let them ask for more — that's the sign it landed.
- **Anything negative about a site or a leader.** Say what the operation needed, never who failed it.

Prepared as an interview value package. Fill the blanks, rehearse the opening and the closer out loud, and cap every behavioral answer at two minutes.